8	Fundal palpation Place the hands on the side of fundus and around the top of the uterus.	Round, hard regular mass – head . Irregular, less firm and not well defined – buttocks
9.	Lateral palpation: Place the hands on both sides of the uterus midway between symphysis pubis and the umbilicus.	A firm continues smooth curvature - it indicates back or spine of fetus. An irregular felt (nobby) parts – fetal extremities
10.	Pawlik's grip: Grasp the portion lower abdomen above the symphysis pubis between thumb and middle finger.	Fetal head above the brim – head is movable. Fetal head not movable – engaged head.
11.	Pelvic palpation:	To determine the level of engagement
12.	Auscultation: Place the fetoscope over the convex portion of the fetus close to the anterior uterine wall.	To hear the fetal heart sounds.
13.	Inform the mother about the findings and make her comfortable.	It promotes confidence.
14.	Replace the articles and wash hands.	Helps in further planning.
15.	Record the findings in the patient's chart (the time, findings and remarks.)	

MEASUREMENT OF UTERINE INVOLUTION

Definition: Involution

Assessing the state of the uterus in post-delivery period as it returns to pre-gravid state.

Purposes

- To identify abnormality
- To rule out infection.
- To estimate the rate of uterine involution take place.

Equipment Required

Screen, Inch tape, Clean gloves.

Procedure

Nursing action	Rational/ scientific principle
Explain the procedure to the patient.	Enhances cooperation Save time and energy.
Assemble necessary articles at bed side.	
Ensure the bladder is empty.	Full bladder may cause upward displacement of uterus
Drape the client exposing only lower abdomen.	provides comfort to the women
Locate fundus with palm of one hand.	Support and stabilize uterus.

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